

Risk Summary

Preeclampsia

Medium Score: 0.6395082154771051

Model Interpretation: Risk-based triage only (not diagnosis).

Macrosomia

Medium Score: 0.5943816940585895

Model Interpretation: Risk-based coaching support.

Summary: The patient, at 32 weeks gestation, presents with a medium risk for preeclampsia, characterized by elevated systolic blood pressure (142 mmHg) and proteinuria. A medium risk for macrosomia is also noted, influenced by a high pre-pregnancy BMI and glucose levels that warrant monitoring, despite not having a GDM diagnosis.

Patient Snapshot (Recent Inputs)

Parameter	Value	Notes
Blood Pressure	142/79 mmHg	Hypertensive threshold ≥140/90, severe ≥160/110.
Proteinuria (dipstick)	1	≥1+ supports PE suspicion in context of HTN.
Symptoms	Headache=0, Vision=0, RUQ Pain=0, Swelling=0	Severe features increase escalation priority.
Glucose	Fasting=99 mg/dL 1hr PP=126 mg/dL	Targets typically fasting <95, 1hr <140 (pregnancy goals).
HbA1c	5.6	Higher HbA1c indicates sustained glycemic elevation.
Maternal Context	Age=38 BMI=42.4 GDM=0	Risk modifiers for fetal overgrowth + metabolic risk.
Activity / Sleep	Post-meal walk=29 min Sleep=6.1 hrs	Lifestyle factors influence glycemic response.
Food Log	paneer salad	Used for dietary correlation patterns.

Reason for Alert

Action Requested: ALERT_DOCTOR

Clinical Rationale: URGENT REVIEW REQUIRED: Patient ID TEST_MED_MED, 32 weeks gestational age. Preeclampsia risk tier is MEDIUM (score 0.64). Patient presents with elevated systolic BP 142 mmHg and proteinuria (dipstick 1). Diastolic BP is 79 mmHg. No reported severe headache, vision changes, upper abdominal pain, or swelling of face/hands. Macrosomia risk tier is MEDIUM (score 0.59) with pre-pregnancy BMI 42.4 kg/m^2, fasting glucose 99 mg/dL, postprandial 1hr glucose 126 mg/dL, and HbA1c 5.6%. Immediate medical evaluation is recommended to assess for preeclampsia and to review glucose management strategies.

Suggested Next Steps

- Review trend: BP, symptoms, proteinuria, glucose readings
- Consider confirmatory tests (protein/creatinine ratio, LFTs, platelets) if clinically indicated
- Consider GDM diet review or insulin adjustment if glucose targets exceeded
- Provide patient education + return precautions

